

Education: Patient, Family, and Caregiver

CMT: CMT-0010 (SR)

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Introduction

Patient education is a component of most standards of care that is emphasized by accrediting organizations as a means of maximizing wellness, improving patient satisfaction, and reducing healthcare costs.(1)(2)(3) The goal of patient education is to help the patient learn within a framework that is organized, well paced, and individualized to meet the needs, interests, and abilities of the patient.(2)(4) A systematic approach to patient and caregiver education includes assessment, planning, implementation, and evaluation.(2)(5)

Many adults have difficulty understanding and using health information. As a result, these individuals have less knowledge of their conditions and treatments, fewer self-management skills, poor adherence, and more medical or medication treatment errors. They also lack the skills needed to successfully negotiate the healthcare system, have less access to healthcare services, and incur increased healthcare costs.(6) Education is more successful for an adult if the learner is self-directed, recognizes the significance of what is taught, finds it personally meaningful and immediately useful, and can use personal experience as a learning resource.(1) For a child or adolescent, developmental level should be considered to appropriately individualize educational plans. In addition, cultural competence increases the success of education by accommodating diverse patient populations.(5)

This tool contains a discussion of the essential components of each of these phases. The importance of including the family and caregiver appropriately in educational activities is emphasized.

Assessment

The purpose of assessment is to gain information about the factors that affect the patient's or caregiver's ability and motivation to learn and use information, so that an effective education plan can be developed. Factors that affect education include individual characteristics, as well as cultural, social, environmental, and condition-related and treatment-related concerns. In addition to the patient or caregiver, other sources for the assessment include the medical record and professionals involved in care.

Learner Assessment

- Identify physical, psychological, and sociocultural characteristics of learners(1)(2)(5):
 - General physical and mental condition (eg, vision impairment, pain level, depression)
 - Cognitive abilities, including focusing, processing, and retaining information
 - Cultural background
 - Health literacy
 - Reading level
 - Language barriers[A]
 - Learning style (eg, auditory, visual, tactile)
 - Attitude, motivation, and readiness to learn(8)
 - Degree of perceived autonomy over individual health practices

- Determine if patient is interested in participating in care.
- Assist patient to set personal goals and outcomes.
- Enhance motivation by helping patient to see meaning in educational activity.
- Seek information about how patient coped in the past.
- Identify level of past adherence or nonadherence.
- Reinforce desirable behavior and results.
- Employment status, occupation, or school enrollment
- Frequency and quality of social relationships, resources, and support(9)
- Educational needs, concerns, and expectations
 - Needs important to learner (eg, pain, finances)(8)
 - Current understanding about health, coverage, and building on existing knowledge
 - Independent research patient may have
- Readiness for change

Table 1. Stages of Change: This table shows a model for how individuals move through stages of readiness. Note that stages do not occur in progression, and a person can move from one stage to any other over time.(2)(10)

Stage	Characteristic	Intervention
Precontemplation	• "I won't." Not thinking about required behavior change (eg, smoking cessation)	• Provide information. • Help patient identify personal priorities and lifestyle goals.
Contemplation	• "I might." Seriously thinking about changing behavior, but has not yet made commitment	• Focus on motivation for change. • Build motivation by having patient review pros and cons of risk behavior and link cons to lifestyle goals.
Preparation	• Decision-making stage, involving plans to begin new behavior (eg, "I will quit smoking on Tuesday.")	• Encourage plan. • Increase commitment to risk behavior change by supporting patient's plan, reviewing counseling resources for teaching coping skills, and negotiating quit date.
Action	• "I am." Notable, overt efforts to change 0 to 6 months after behavior change begins	• Support behavior change. • Check availability of social support. • Review patient's skills for managing situations that can trigger relapse.
Maintenance	• "I have." Change is sustained over time.	• Prevent relapse and encourage long-term change. • Review factors in patient's life that support new adaptive lifestyle habits, and reinforce patient's achievement of targeted lifestyle goals.

Educator Assessment

- Educator should assess their individual feelings and responses to patient or content being taught.(1)
- Purpose is to determine if personal issues or beliefs may interfere with transmission of material, or interaction with patient or caregiver.
- Seek supervision, consultation, or referral to another professional if educator's responses make it impossible to educate effectively. (1)

Environment

- Plan time and place for education.(1)
 - Choose safe, comfortable, and private location.
 - Make sure all educational materials are at hand and ready for use or distribution to participants.
- Include others as needed and desired.(1)(11)
 - Involve family members and caregivers to support care of patient.
 - Assess family and caregivers as described in assessment, and include them in subsequent educational activities.

Planning

A formal teaching plan should be developed when there is a specific need about health promotion or management of a specific health problem. A planning process is useful for guiding and communicating the educational process and consists of identifying outcomes and methods.(1)

- A formal written plan is useful to:
 - Organize teaching.
 - Provide point of reference for patient.
 - Use when evaluation takes place. Update plan to meet changing learning needs.
 - Document process that has taken place.
 - Communicate with other members of interdisciplinary team.(12)
- Learning goals need to be clearly stated.

Outcomes

When assessment is completed, plan desired outcomes. Create realistic goals that align with assessment information. These goals must be practical, attainable, relevant, and culturally sensitive. Core beliefs and value systems are held more strongly when people lose control over aspects of their life due to illness. Consider the following factors(1)(13):

- Identify measurable and observable outcomes aligned with objectives.
- Incorporate behavior into patient's lifestyle.
- Determine time frame for teaching and evaluation.
- Evaluate whether patient and caregiver have all needed materials, are able to demonstrate understanding of what has been taught, and can perform needed healthcare.
- Create plan for managing relapse, which is common in process of behavioral change.

Methods

Methods are teaching techniques used to achieve educational outcomes.

- Comprehensive interventions (combining cognitive, behavioral, motivational, affective, and psychomotor components) that establish the most effective way of communicating with patient; effective strategies include(1)(13):
 - Translate medical terminology to simpler words.
 - Modify written materials. Examples include:
 - Use large print and boldface.
 - Use short sentences.
 - Use pictures or symbols.
 - Provide easy-to-see and understandable visual aids such as anatomical pictures or models, textbook pictures, or patient-teaching materials.
 - Limit presentation to essential information, emphasizing most important points first.
 - Write information at a sixth-grade level (or lower), including:
 - Medical instructions
 - Medication labels
 - Appointment slips
 - Explanation of chronic condition
 - Use patient's primary language to convey information. Examples include(2)(7):
 - Provide written materials in primary language of patient or caregiver.
 - Use interpreter if needed.
 - Select interpreter who is specifically trained in healthcare interpretation.
 - Select interpreter who meets cultural needs of patient (eg, in some cultures it is inappropriate to discuss personal, health-related information with someone younger or of the opposite gender).
 - Establish relationship with patient in presence of interpreter:
 - Speak directly to patient when asking questions or giving instructions.
 - Ask patient to speak directly to you.
 - Ask interpreter not to edit information.
 - Note nonverbal responses in addition to spoken language.
 - Learn as much as possible about patient's culture and beliefs to aid in understanding and interpreting information.
 - Give patient option of using family members or friends, if this enables more unreserved exchange of information or if patient is more comfortable sharing personal information.

Implementation

Implementation involves actual teaching of content through defined process goals that focus activity and allow for problem solving in real-life situations, and methods that describe key elements of successful education.(1)

Goals for Implementation

- Establish trust.(11)
- Pace content to patient's ability to concentrate, and allow opportunity to ask questions.
- Support self-efficacy, relying on patient choices.(14)
- Provide relevant information.(8)
- Involve patient and caregiver.
- Manage health system processes (eg, how to get prescriptions refilled, when to follow-up with primary care provider).(15)

Methods

- Implement methods that are relevant to and appropriate for patient according to assessed characteristics; examples include(1)(2):
 - Break concepts into components.
 - Do not present all information at one time (eg, limit instruction to 3 to 4 points per session) and schedule several sessions to cover complex material and check on previous learning.
 - Build in review, demonstration, feedback, and opportunity to ask questions.
 - Use teach-back approach, asking patients to explain what they have been told.
- Schedule time and place that works best for patient, minimizing distractions.
- Speak clearly, slowly, with low pitch, and vary tone of voice.
- Face learner at eye level.
- Build information on prior knowledge.(6)
- Use preferred learning style (ie, auditory, visual, tactile).
- Avoid argument or moralizing.
- In calm, neutral tone of voice, identify behaviors that may indicate resistance (eg, procrastination, frequent forgetfulness of self-management activities).
- Discuss what to do if part of regimen is inadvertently neglected (eg, missed dose of medication).
- When questions arise that cannot be answered, help obtain information.
- Schedule follow-up to review understanding(5)

Evaluation

Evaluation should take place throughout the teaching and learning interaction, and appropriate modifications made. In the evaluation phase, measure progress toward specific goals defined during the planning phase. In addition, use the following indicators to determine value from an educational experience(1)(13):

- Educational plan and process is modified as needed according to abilities of patient.
- Patient and caregiver possess understanding of and skills required to accomplish tasks.
- Patient goals are met.
- Health outcomes specific to condition are achieved.(5)
- Patient and caregiver express satisfaction with information gained and how it was achieved.
 - Anxiety controlled
 - Caregiver performance satisfactory
 - Positive perceived ability to perform tasks
 - Positive participation in healthcare behaviors

Documentation

Documentation is a professional responsibility and is important to the education process. It supports progress toward goals over time and provides information useful to other caregivers. Document with standardized patient teaching checklists(1):

- What was taught and methods used
- How learners responded

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Footnotes

[A] Instructions should be given in patient's, family's, and caregiver's native language using trained language interpreters whenever possible.(2)(7) [A in Context Link 1]

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